

HPAP-200 Donor Summary



HPAP	200	UNOS	
Recovery OPO	GLDP Gift of Life Donor program	Allocation Via	UPENN ☒ nPOD ☐
Age (years)	51	DCD	YES ☐ NO ☒
Race	Caucasian	DBD	YES ☒ NO ☐
Sex	M ☒ F ☐	Admission to Cross Clamp	132 Hours 29 Mins.
ABO (Rh)	O positive	Cross Clamp Time	10/23/2025 12:29 EDT
BMI (Kg/m ²)	26.63	Cold Ischemia Time*	2 Hours 51 Mins.
Weight (kg)	81.8		
Height (cm)	175.26		
Cause of Death	Anoxia	Preservation Solution	UW ☒ HTK ☐ UW/Belzer Cold Storage/Viaspan/SPS-1
Mechanism of Injury	Drug Intoxication	Organs Recovered	Heart ☒ Kidney ☒ Lung ☒ Pancreas ☒ Liver ☐ Intestine ☐
Cardiac Arrest/Downtime	Yes ☒ No ☐	Intraoperative time lapse from liver to pancreas removal from the peritoneal cavity:	Hours Mins. NA
CPR / Time	Yes ☒ No ☐ Time Not Reported	Organs Discarded	Heart ☐ Kidney ☐ Lung ☐ Pancreas ☐ Liver ☐ Intestine ☐
Total Est. Downtime	Not Reported	Blood Culture	NA
Date /Time of Admission	10/18/2025 00:00 EDT	PHS High Risk	YES ☐ NO ☒
		Acute Lung Injury	Improving lower-lung opacities, likely atelectasis and post-bronchoscopy changes, with possible mild inflammation.

*Cold Ischemia time is calculated from time of cross clamp to start of enzyme perfusion for islet isolation



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Medical History:

		Duration	Medications	Compliance
Type of Diabetes	None	-----	-----	-----
History of cancer	-----	-----	-----	-----
CAD	-----	-----	-----	-----
Hypertension	yes	2 months ago	Amlodipine 5 mg	yes
Hyperlipidemia	HLD	-----	-----	-----
Autoimmune disease	-----	-----	-----	-----
Family History	CAD ☐	Diabetes ☐	Auto immune disease ☐	Others:
Surgical History:	Cervical fusion (2012); hernia repair (1996); appendectomy (childhood); vertebral fracture repair.			
Comments:	Medical Hx: Asthma, lumbar DJD, GERD, Possible left kidney infarct in past (Historical Cr 1.0), Gout, Bipolar/ADHD, allergies to Coconut oil, Indocin, Tizanidine Medication Hx: Bupropion 150 mg, Vyvanse 20 mg, unknown Psych meds for 10 years, was on Eliquis has been D/C, Albuterol 10 years Social Hx: Former smoker 1ppd cigarette for 25 years and Quit 6 years ago, daily vape			

Hemodynamic Profile

Average BP During Hospitalization (mmHg)	148/87	
Average Low BP During Hospitalization	124/73	Duration: 10 min.
Average BP in OR (mmHg)	130/82	
Average Low BP in OR (mmHg)	88/60	Duration: 60 – 169 min.
Average HR in OR (bpm)	92	
ABG-pH range	7.1 – 7.43	

INTERVENTION

Blood Products/Meds Transfused Before Organ Recovery

Product	Amount (ml)	Units	Total (ml)
Fresh Frozen Plasma	-----	-----	-----
PRBCs	-----	-----	-----
Platelets	-----	-----	-----
Norepinephrine (Levophed)	-----	0.03 mg/kg/min started 121:46 hrs. before organ recovery for the duration of 54:27hrs.	
Vasopressin	-----	0.01 units/min Started 53:21 hrs. before organ recovery for the duration of 3:47hrs.	
Neo-Synephrine	-----	-----	-----
Epinephrine	-----	5-8 mcg/min started 121:57 hrs. before organ recovery for the duration of 3:31Hrs.	
Phenylephrine	-----	-----	-----
Dopamine	-----	-----	-----

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Blood Products/Meds Transfused Intraoperative

Product	Amount (ml)	Units	Total (ml)
Fresh Frozen Plasma	-----	-----	-----
PRBCs	-----	-----	-----
Platelets	-----	-----	-----
Norepinephrine (Levophed)	-----	-----	-----
Vasopressin	-----	-----	-----
Neo-Syneprine (phenylephrine)	-----	-----	-----
Epinephrine	-----	-----	-----
Dopamine	-----	-----	-----
Heparin	-----	30,000	-----

Initial Autoantibody Screening (nPOD): ELISA

Not performed for HPAP-T2D program

GAD-65	IA-2
ND	ND

Confirmatory results: Radioimmuno Assay (RIA)

	GAD-65 (unit/ml)	IA-2 (unit/ml)	Insulin AAB (unit/ml)	ZnT8 (unit/ml)
Results	0	0	0.001	-0.003
Cut-off values	20	5	0.01	0.02

*Sample obtained at time of organ recovery.

	C-peptide (ng/ml)	Proinsulin
Results	5.71	ND

*Sample obtained at time of organ recovery.

HLA (OPO)*

Class 1	A	1	24	Class II	DR	17	4
	B	8	61		DR51	N-Negative	N-Negative
	C	07	15		DR52	52	N-Negative
	Bw4	Negative			DR53	53	N-Negative
	Bw6	Positive			DQB1(DQ)	2	5
			DQA1		01	05	
			DPB1		01:01	01:01	
			DPA1		02	02	
Comment:							

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Confirmatory HLA (UPENN)*

Not performed for HPAP-T2D program

Class 1	A			Class II	DRB1		
	B				DRB3		
	C				DRB4		
					DRB5		
					DQB1		
					DQA1		
					DPB1		
					DPA1		

*HLA typing performed using NGS

Infectious Disease Serology

Test	Result	Hemo/Plasma Dilution Status	
		Qualified	Non-Qualified
EBV IgG	Positive	✓	-
EBV IgM	Negative	✓	-
CMV (IgM)	Negative	✓	-
CMV (IgG)	Negative	✓	-
HBcAb	Non - Reactive	✓	-
HBsAg	Non - Reactive	✓	-
HCV Ab	Non - Reactive	✓	-
HIV I/II (HIV Ag/Ab Combo)	Non - Reactive	✓	-
Syphilis	Non - Reactive	✓	-
Procleix Ultrio	NA	-	-
Ultrio HBV	Non - Reactive	✓	-
Ultrio HCV	Non - Reactive	✓	-
Ultrio HIV	Non - Reactive	✓	-
Toxoplasma Ab	Negative	✓	-
SARS-CoV-2	Negative	✓	-

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Laboratory Panel

	Initial	Peak	Terminal
Na (mEq/L) (135-145)	139	159	152
Creatinine (<1.5)	1.46	2.23	1.48
Glucose (mg/dL) (60-150)	348	348	153
HbA1C%	6	----	----
Total bilirubin (0-1.0)	0.2	0.6	0.5
SGOT (AST) (0-40)	105	325	29
SGPT (ALT) (5-35)	119	208	38
Alkaline phosphatase (45-110)	111	166	166
Serum Amylase (23-851)	44 (10/20/25)	74	74
Serum Lipase (0-80)	27 (10/20/25)	29	13
WBC (THO/uL) (4.5-11.0)	8	24.1	15.7
Hgb (g/dL) (12-16)	10.1	11.7	8.8
Platelets (THO/uL) (150-350)	342	542	234
INR (<2.0)	1.1	1.3	1.3

Urinalysis

	1 st	2 nd	3 rd	4 th & 5 th
Glucose	Negative	Negative	Negative	Negative

Medications During Hospitalization

Steroids**	Methylprednisolone 40 mg		
Diuretics	Lasix 100 mg, Mannitol 50 gm		
T3 Protocol	NA		
T4 Protocol*	-----		
Insulin**	Insulin-Regular 2 units		
Antihypertensive	Diltiazem 20 mg		
Vasodilators	Diltiazem 20 mg		
DDAVP**	NA		
Total parenteral nutrition	NA		
Other	Amiodarone 1mg/min, Precedex 0.05 mg/kg/min, Diltiazem 7.5 mg/hr., Fentanyl 5mcg/min, Azithromycin 500mg, Amiodarone 150mg, Keppra 1000mg, Lamictal 200 mg, Lovenox 40 mg, Doxycycline 100mg, Ceftriaxone 1gm, Albutrol 3 ml, Versed 1 mg, Magnesium Sulfate 2 gm, Cefepime 20 mg, Potassium Chloride 20mEq, K+ bicarb-citric acid 40 mEq	Specify	

*T4 protocol: Levothyroxine, (20 mcgs), Solumedrol (2 gms,) Dextrose 50%, (1 amp), Regular Insulin (20u), Vasopressin (1 unit)

Mi Z, Novitzky D, Collins JF, Cooper D KC. The optimal hormonal replacement modality selection for multiple organ procurement from brain-dead organ donors. Clinical Epidemiology 2015;7 17-27.

** Excluding T4 Protocol